PREPARED FOR **the Forus team**

About a 4-minute read

Forus provider onboarding — two sign-ups, two outcomes

I signed up with the same information, changing only the specialty. Ophthalmology took 59 seconds and ended in a waitlist. Dermatology took 3 minutes and 10 seconds and gave me access to the provider portal.

A few things are working well: specialty-based routing, the personal waitlist follow-up, and the ability to queue prescriptions before verification finishes.

I also found five opportunities worth investigating. These are outside-in observations, not conclusions about your actual funnel. I'd be interested in where your data agrees or tells a different story.

Carlos Duchicela · Growth product leader November 2026

Based on two complete runs of the live provider flow, Nov 9, 2026

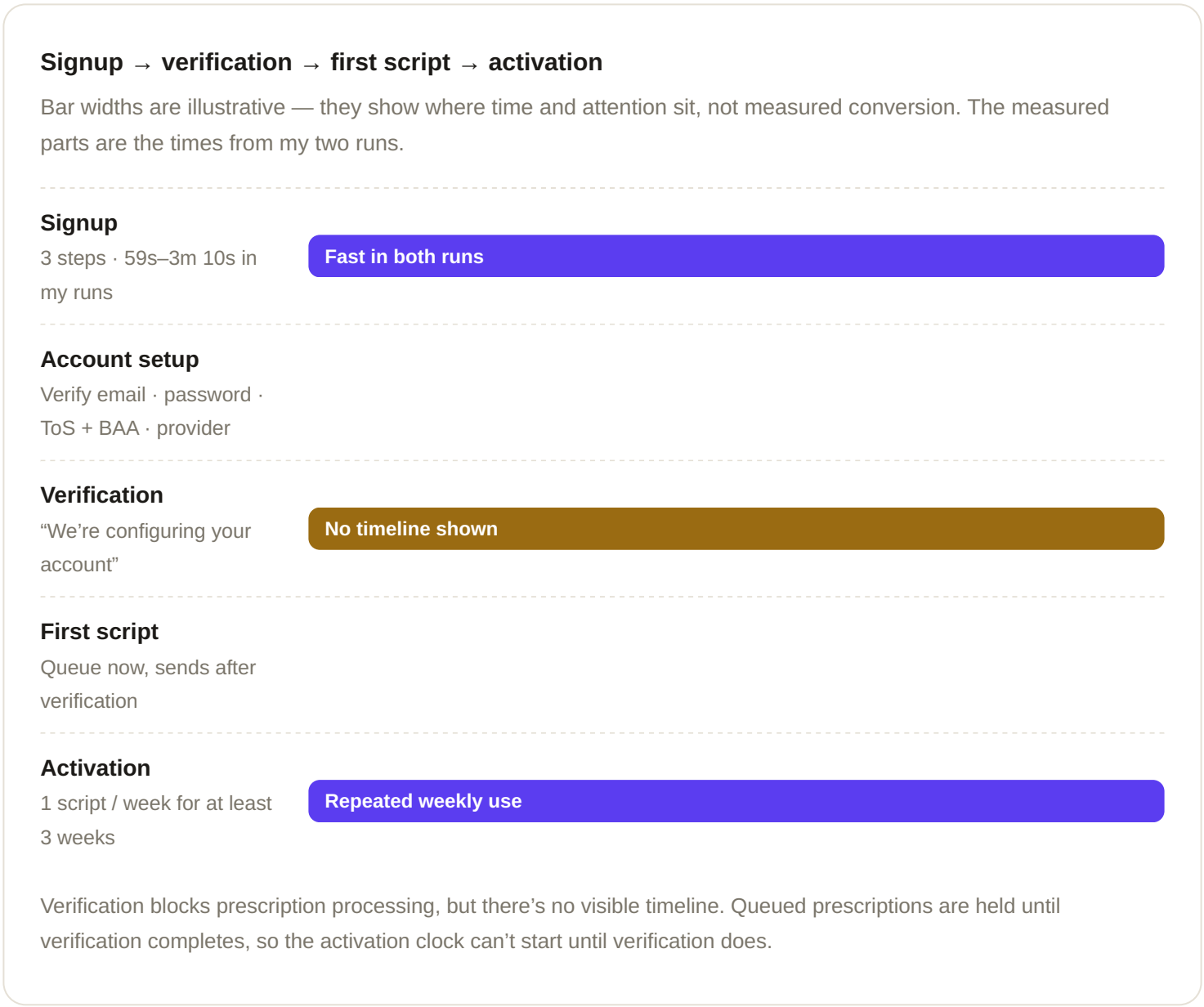
1 · WHAT I DID

Two runs, two specialties

Run 1 (Ophthalmology) took 59 seconds: three steps, then a waitlist screen. Taylor's email asking for the medications my practice most often prescribes that need prior auths arrived afterwards, by email.

Run 2 (Dermatology) took 3 minutes, 10 seconds: email verification, password, Terms of Service and BAA, provider setup, and a working dashboard.

The difference is whether the specialty is live. That's deliberate sequencing, and I'd keep it.



Activation is sending 1 script per week for at least 3 weeks. That replaced the previous metric, the % of doctors sending 10 scripts in 30 days.

I don’t know if verification is the biggest source of drop-off — I don’t have your funnel data. It’s the clearest gap I could observe, and I’d want to see the funnel before prioritizing it.

Three milestones worth keeping separate:

A

First queued / submitted script

The first meaningful action. A practice has done the work.

B

First successfully processed case

Value realized. A case actually goes through.

C

Repeated use

1 script per week for at least 3 weeks — activation, as defined above.

Open question: does Forus count queued prescriptions toward activation, or only prescriptions submitted after verification? This is n = 2, from the outside. Where I assert something, I watched it happen on screen. Where your data would settle it, I ask in the finding.

2 · WHAT'S WORKING

Three choices I'd keep

- **The specialty branch.** I like the specialty branch. It prevents practices from getting into a product that isn't ready for them. I'd keep that.
- **The personal waitlist follow-up.** Taylor's email asks waitlisted practices for the medications they most often prescribe that need prior auths, "so we can hit the ground running." It's signed by a person, and it turns waiting time into useful setup. Today it lives in a blank email instead of the product.
- **"Queue up prescriptions."** A practice can queue prescriptions right away — "We'll hold them until your account is verified" — so work can start before verification finishes.

Smaller signs: "Biologics Coordinator / PA Specialist" is in the role list, training books with a named human (Ashalia Dawkins), and signup asks "How did you hear about us?" with "Someone from another practice" as an option.

3 · FINDINGS

Five findings

Smaller things are left out. These five are the ones I'd investigate first.

Finding 01

Make verification status visible

Practices can finish setup without knowing how long verification will take or whether anything is required of them.

What I saw. After the BAA and provider setup, the dashboard says “We’re configuring your account” and “Complete verification to start processing PAs.” No timeline, no progress, no next action. The Tasks view is empty. A practice finishes setup and immediately loses visibility into what happens next.

WHAT I’D TRY

- Show the current state, what’s being checked, and any outstanding actions.
- Add an estimated window only if the data supports one.
- Keep “queue up prescriptions” visible during the wait.

- **Measure:** verification completion, elapsed time, and first-prescription conversion. **Question:** what’s the verification timeline today, how variable is it, and what share of practices never complete it?

Finding 02

Give new practices a clear first step

The dashboard offers six setup actions without a clear priority. Once you’re inside, onboarding is a menu — not a path.

What I saw. Six cards — add providers, add office address, queue prescriptions, notification preferences, invite teammates, add office numbers — with no order and no progress. None shows whether it’s done, pending, or blocked.

There’s also a role question here. The person who signs up — often the physician — may not be the person who runs prior authorizations day to day, often a coordinator or PA specialist. A guided path should account for who actually does the work.

WHAT I’D TRY

- Guide each user toward the next action needed to process their first prescription, one step at a time, with progress.
- Keep the other settings accessible underneath, each labeled done, in progress, or blocked.

- **Measure:** time to first prescription and repeat weekly use. **Question:** what should a new practice do first once inside — and is the menu deliberate?

Finding 03

Improve referrals and team invitations

Practice referrals open a separate HubSpot form. Team invitations are buried in Settings.

What I saw. “Refer a practice” opens a HubSpot form on a third-party domain, still says “formerly Tandem,” asks for every field by hand, and defaults Specialty to “Allergy and Immunology” — the field the whole flow branches on. “Invite your teammates” is the fifth of six dashboard cards, its button says “Go to Settings,” and the form asks for name, email, Permissions, and Role with no explanation of what any role can do. Afterwards, neither flow shows status: a referrer never learns whether their referral signed up.

WHAT I’D TRY

- Bring referral in-product: referrer pre-filled, referred practice looked up by name, no defaulted specialty, attributed to the referrer.
- Surface the referral ask after a practice has seen value — after a first processed case, not at signup — and show status: invited → signed up → activated. Recognition and visible status are enough; no reward should change how fast any practice gets verified or gets care.
- Make invite a real flow: an Invite button that invites, each role explained in one line, and invite state on the Users page.

- **Measure:** qualified referrals, referred-practice activation, and team adoption. **Question:** how is a referral attributed today, and does a referrer ever hear back?

Finding 04

Look up provider information instead of asking users to type it

Fetch, don’t ask. The NPI step required me to leave Forus to find a provider identifier.

What I saw. Provider setup asks for a Provider NPI Number and validates by checksum only. 1234567890 and 0000000000 fail; a real, publicly listed NPI — 1558704544, for a skilled nursing facility — was accepted. Nothing explains what an NPI is or where to find yours, so I left Forus and searched the public NPPES registry to finish the step. The check confirms the number is well-formed, not that it belongs to the person entering it.

Identity needs care here. An organization has a Type 2 NPI; an individual prescriber has a Type 1 NPI; staff are neither. NPPES can prefill details, but a match does not prove someone is authorized to act for a practice. Identity and authorization checks should stay separate.

WHAT I'D TRY

- Add an NPPES lookup on the provider-setup screen — search by provider or practice name, pick the record, prefill practice name, address, and specialty.
- Keep a manual fallback, and keep any identity or authorization check as its own step.
- Apply the same idea elsewhere, including a structured medication-list capture for waitlisted practices instead of a blank email.

- **Measure:** provider setup completion and time spent on the step. **Question:** is the checksum-only check deliberate, with identity matched downstream?

Finding 05

Simplify the forms

Signup and setup are dense, with little indication of progress.

What I saw. Long paragraphs where a short label and one line of help would do, everything presented at once. No progress indicator — no "step 2 of 3," no bar — and nothing to suggest progress is saved if you stop midway. The clinical steps — BAA, NPI, verification — can't be removed. The form around them doesn't need to feel like paperwork.

WHAT I'D TRY

- Shorten instructions, group related fields, and chunk the steps.
- Make progress and recovery clear: a progress indicator, inline validation, autosave, and a resume link for anyone who drops off.

- **Measure:** per-step completion, abandonment, and downstream activation. **Question:** do you measure drop-off per step or per field today?

How this maps to the role

Activation — Findings 1, 2, and 5.

Referral — Finding 3.

Metrics — the funnel and milestones in section 1.

Briefly on background: at PrizePicks I ran Player Accounts, a trust-and-compliance-gated onboarding funnel (identity, KYC, payments) — +25% registration-to-first-deposit, +5% KYC pass rate. At Superhuman I rebuilt growth strategy around the behaviors that predict retention and activation. At WeTransfer I built a commerce path — pricing, packaging, onboarding, checkout — from zero, +35% conversion.

5 · THE FIRST 30 DAYS

Start with the evidence

I wouldn’t ship against a guessed failure point. First, find where the largest actionable loss actually is.

DAYS	WORK	WHAT IT DECIDES
Days 1–5	Instrument eligibility, verification, queued / submitted, processed, and repeat-use transitions, broken out by specialty and by role.	Where the largest actionable loss occurs.
Days 6–10	Shadow 5–8 relevant staff. Review exceptions, drop-offs, and support reasons.	Why the loss happens, in the words of the people doing the work.
Days 11–20	Ship or simulate one bounded intervention at the verified point of failure.	Whether a focused change moves the transition.
Days 21–30	Evaluate repeat use, cost, verification integrity, and safety.	Scale, revise, or stop.

Six questions

- **Sequencing.** What orders the specialty sequence — EHR readiness, practice size, referral source, payer coverage? Is there anything a waitlisted practice can do to move up, beyond sending the medication list?
- **The email wall.** Every path asks for a work email before showing the product. Is that deliberate qualification, and how do you weigh its cost in the evaluators who bounce off it?
- **Training.** Is the training call with Ashalia required before a first script, or an accelerator — and what does time-to-first-script look like with and without it?
- **Who does the work?** In practices that become consistently active, who does the work day to day — doctors, MAs, admins, or PA specialists — and does the first-run flow reflect that?
- **Organization vs. individual identity.** How do you distinguish organization identity (Type 2 NPI) from individual prescriber identity (Type 1 NPI), and who is authorized to act for a practice?
- **BAA authority.** Who can sign an organization-level BAA, and how is that authority validated?

I ran your onboarding because Madeline suggested I try it and bring questions, and I came away more interested in Forus, not less. I'd love to discuss where my read matches yours, where your data corrects it, and where you've already made the opposite call for reasons I can't see from outside.

— Carlos

Carlos Duchicela · November 2026

Written from two hands-on runs of the public provider flow (Nov 9, 2026). Happy to walk through the recordings together.